ENSIGN SERVICES

POSTette: The Rehab Evaluation

Reference: Rehabilitation P&P Policy 220- The Evaluation and Plan of Care; Medicare Benefit Manual Chapters 8 & 15

Capture how the patient is performing certain activities, skill sets, and tasks at the time of that evaluative treatment encounter. Consider the medical diagnosis (reason for referral) and what limitations/impairments are exhibited that impact function. It is better to have a clear concise evaluation with emphasis on the deficits that will be addressed in treatment rather than collecting data that does not tie back to anything functional or practical for treatment intervention.

Establish baselines using objective data, **standardized tests**, measurements, and descriptive clarification. When using standardized tests, be sure to indicate scores and what those scores mean.

- Gather objective data via direct observation of patient performance of functional tasks
- Complete standardized assessments and be sure to include the scores as well as the actual standardized test in the Medical Record.
- Obtain/review all patient/family reports, medical reports.
- Establish functional deficits as they relate to the tasks and activities typically performed by the patient.

Medical History and Co-morbidities

When completing the evaluation, make note of the patient's medical history. Determine if any prior or secondary conditions will affect clinical outcomes and be sure to document how and why. Co-morbidities can impact the method of treatment delivery as well as frequency and duration of services. For example, cardiac dysrhythmia alone may not justify the need for skilled intervention; however, this condition can significantly affect the rate of progress. Co-morbidities can also necessitate the need for precautions (oxygen use) or contraindications (pacemaker with modalities).

Skilled Admissions – Therapy Evaluations

- The ONSET DATE for the treatment code is the date of the evaluation (the date the treatment code was determined).
- Medical DX codes are not required for Skilled Payers. Treatment codes can be listed in both areas on the evaluation.

Long-Term Care, Part B & Outpatients

- The onset date for the **Medical Conditions** supporting skilled therapy services (LTC and Outpatients) will be determined by the therapist completing the evaluation after reviewing the medical history and/or records from the physician.
- The onset date for the **medical code** will be ON OR AFTER admission to the SNF for LTC Residents.
- The onset date for the **treatment code** is the date of the evaluation (the date the treatment code was determined).

Prior Level of Function

This is the functional status of the patient prior to the onset of the spell of illness and is discipline specific. Indicate the level of independence in each area of functional performance that will be addressed in the treatment plan (e.g. independent with high level ADL's; independent with ambulation without assistive device; tolerated regular texture with thin liquids). This information helps clarify the reason for treating the patient and provides support for continuation. It also demonstrates medical necessity, sets expectations of functional goals and outcomes, and supports long term discharge plans.

Example: Prior to CVA patient was independent with mobility including negotiating stairs and community ambulation.

Describe patient Strengths along with Weaknesses:

Evaluations also determine the strengths of the patient that support therapeutic intervention. If necessary, break up functional activities to describe a patient's ability to perform those activities. Too much emphasis on weaknesses without demonstrating strengths could lead to documentation denials. This is critical with cognitively impaired patients.

Weakness	Strength
Unintelligible Speech	Intact auditory comprehension Partial control of musculature
Limited Attention Span	Can maintain attention for 3 minutes and follows simple commands
Unsafe in environment	Performs better with structured environment
L Hemiplegic	R hand dominant, R side unaffected

Assessment Category	Content to include on Evaluation
Aerobic Capacity & Endurance (assess vital signs pre and post activities)	• Aerobic capacity during functional activities • Cardiovascular issues with exercise or activity • Pulmonary issues with exercise or activity
Anthropometric Characteristics	• Body composition • Body dimensions • Edema
Cognition, Attention, Arousal	• Consciousness • Orientation to time, person, place • Ability to follow commands (at what level of complexity) • Communication
Cranial & Peripheral Nerve Integrity	• Response to stimuli • Sensation Testing
Pain	• What scale used to assess • Location, description, exacerbating and relieving factors • Impact on function

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Assessment Category	Content to include on Evaluation
Assistive & Adaptive Devices	• Any devices used during functional activities • Components, alignment, fit, and ability to care for the devices • Balance using devices • Safety using devices
Orthotic & Protective Devices	• Safety with or without • Components, alignment, fit and ability to care for devices
Gait & Balance	• Static & dynamic balance with and without devices • Gait with and without devices • Gait / Balance during functional activities • Safety during gait • Gait patterns • Gait / Balance assessments used • Standardized test scores
ROM and Manual Muscle Testing	• Get **specific** ROM measurements for both active and passive (avoid WFL or WNL) • How was manual muscle testing conducted • Muscle flexibility • Joint integrity • Standardized measurements and scores
Wound	• Type of wound • Characteristics such as size, depth, odor, drainage, etc. • Any signs of infection • Activities or positions that will aggravate or relieve the wound
Self-Care	• Ability to perform self-care with or without assistive devices, equipment, etc. • Safety with self-care tasks • Scores from standardized assessments
Communication /Cognition	• Auditory comprehension • Verbal expression • Social interactions/ pragmatics • Speech intelligibility • Problem solving / executive functioning • Scores from any standardized assessments
Swallowing	• Oral, pharyngeal and laryngeal functioning (muscular & sensation) • Textures assessed • Results from MBSS • Scores from standardized assessments
Home Environment	• Patients living situation prior to incident • Does patient live alone? If not who lives with them. • What floor(s) is the bedroom, bathroom, laundry, on? • Number of stairs between floors.

Assessment Category	Content to include on Evaluation
	• What is the patient responsible for at home? • If the patient previously required assistance – will that assistance still be available for the patient's current condition.

Treatment Diagnosis (es)

Clinicians often select a treatment diagnosis before completion of the evaluation. Treatment codes clarify the need for skilled intervention and must relate to the impairments identified on the evaluation. Determine what condition best supports the reason for treatment. The treatment diagnosis must relate back to deficits captured on the evaluation.

For example:

If OT selects "lack of coordination" as the treatment diagnosis, then the evaluation must reflect an impairment of coordination. If the evaluation, however, indicates coordination as intact, a conflict is created and leads to a treatment diagnosis that does not support intervention based on the baseline data. As a rule of thumb is it also recommended avoiding vague and generic treatment diagnoses such as: malaise, fatigue, or debility.

Once all of the objective data is collected and documented it is important to give a brief summary and/or overall clinical impression of the patient.

Evaluation and Treatment on Same Date of Service

- Differentiate treatment vs eval when performed on the same date.
- Documentation needs to support that both were necessary and reflect that the treatment provided was more than gathering of baseline data.
- The documentation must clearly indicate that treatment is separate and distinct from the evaluation.
- Ensure there aren't any "trends" related to minutes provided for evaluations.

Re-evaluations

Re-evaluations are provided during the same course of treatment and are generally not as extensive as an initial evaluation. It is important to note that on-going assessment of a patient's status/progress is considered a component of skilled intervention and does not require a re-evaluation.

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